

steady

A clinician-governed, autonomous trauma-care system —
priced as a daily wellness habit, monetized as clinical software.

INVESTOR DECK SOURCE DOCUMENT

Complete product inventory · activation requirements · differentiation
monetization · growth model · deck-builder handoff

Prepared 29 July 2026 · Confidential

The investment case in one page

Steady is not a meditation app with a trauma skin. It is a deterministic, clinician-ratified care engine that decides — every single day, per member — what is safe for that person to do, and then does it with them. The content is inseparable from the engine that sequences it. That is the asset.



Six reasons this is a good investment

- 1. The hard part is already built and clinically ratified.** Two independent licensed psychologists reviewed and signed off the deterministic safety configuration on 22 July 2026. Competitors sell content anyone can license; Steady sells a governed decision system that took a clinical corpus, a rules engine, and a signed review to produce.
- 2. Full product is live on two platforms today.** Web and native iOS, sharing one backend, one memory bank, one safety engine. A member's calm place, triggers, and progress follow them across devices. Not a roadmap — running now.
- 3. The pricing ladder is engineered, not guessed.** Entry at **\$6.99** undercuts every major competitor (\$9–15) to buy cheap top-of-funnel; the ladder climbs to **\$34.99** for an autonomous care tier none of them offer. Every new member starts on a 7-day Premium trial — they experience the top of the ladder before they ever pay.
- 4. Monetization compounds without new acquisition spend.** A memory-driven upsell engine watches each member's own data and recommends a tier change only when their behavior earns it. Revenue expansion is a product feature, not a marketing campaign.
- 5. Structural retention.** The daily layer (breathwork, meditation, movement, sleep) creates habit; the memory bank creates switching cost that compounds with every session. Leaving Steady means abandoning a system that knows what grounds you at 2 a.m.
- 6. Defensible regulatory posture, deliberately chosen.** Steady launches in the wellness lane — no diagnosis, no treatment claims — with a tamper-evident audit chain, a clinician review console, and a documented sign-off ledger already in place. Most consumer mental-health companies retrofit governance after an incident. This one started there.

Disclosure — read before using these figures

Steady is a **pre-revenue development prototype** with a live investor demo and **no production users to date**. Every product count, test count, and capability claim in this document is verified against the shipping codebase. Every financial figure is a **model with stated assumptions, not a result** — sections containing assumptions are marked. Competitor pricing is drawn from public plans and should be re-verified against live pages before the deck is presented. Nothing here should be represented as traction.

The problem, and the gap in the market

For the person

EMDR is one of the most strongly evidenced treatments for PTSD — recommended in frontline clinical guidelines from the World Health Organization, the UK's NICE, and the U.S. Department of Veterans Affairs. But access is brutal: in-office sessions run **\$150–350 an hour**, waitlists run months, and the format requires telling a stranger the worst thing that ever happened to you. Most people who would benefit never start.

Meanwhile the apps they *can* afford — Calm, Headspace, Insight Timer — offer general relaxation content with no clinical structure, no screening, no pacing, and no idea whether today is a safe day for that particular person to go anywhere near a difficult memory.

For the market

The category has split into two poles, and the middle is empty:

Category	Examples	Typical price	What's missing
Content libraries	Calm, Headspace, Insight Timer, Balance	\$9–15/mo	No clinical structure, no screening, no per-day safety decision, no accountability. Content is licensable — no moat.
Coaching / therapy marketplaces	BetterHelp, Talkspace, Sanvello (coaching tiers)	\$60–400/mo	Human-capacity-bound, so margins compress with scale; expensive; still waitlisted.
The gap Steady occupies	—	\$6.99–34.99/mo	Software-margin economics with clinical structure: screening, daily gating, deterministic safety rules, specialist oversight, and an autonomous care loop.

The one-sentence positioning

Calm helps you relax. A therapist helps you heal but costs \$200 an hour and has a three-month waitlist. **Steady runs a structured, clinician-governed trauma program with you — every day — for less than a streaming subscription.**

Why the timing is right

- **Consumer price ceiling has been established** — a decade of Calm and Headspace taught a mass market to pay a monthly subscription for mental wellbeing. Steady does not have to create that behavior, only redirect it.
- **Deterministic AI safety is now a requirement, not a differentiator** — the market and regulators have turned sharply against "LLM as therapist." Steady's architecture, where the model is structurally prevented from making any safety decision, is the posture the category is being forced toward.
- **Remote EMDR has published RCT support** — including a multisite randomized controlled trial across 10 hospitals showing large reductions in PTSD, anxiety, and depression delivered fully remotely.

Everything Steady provides — complete inventory

All items below are implemented and running on web and iOS unless explicitly tagged otherwise. Counts verified against the shipping codebase on 29 July 2026.

3.1 The safety substrate — every tier, never gated, never sold

This layer is the product's spine. It runs beneath all three tiers, and access to it is never restricted by payment status — a deliberate, documented invariant.

Capability	What it does	Status
Program-fit screener	8 mandatory yes/no items. Any hard-stop answer (self-harm, recent hospitalization, psychotic/dissociative diagnosis, substance dependence, unsafe situation, under 18) closes all sessions, routes to crisis resources, imposes a 24-hour cooldown, and auto-refunds the billing period . Clinician-ratified wording.	LIVE
Daily check-in	Under 90 seconds. Deterministic first-match routing into crisis / grounding-only / stabilization / processing-OK, and recalculates the readiness score daily.	LIVE
14-step access gate	checkModuleAccess evaluates kill switch, subscription, tier, consent, screener, measures, profile, today's check-in, routing, readiness track, safety plan, prerequisites, specialist unlock, daily cap, and SUDS cooldown — in that order, first failure blocks with a member-visible reason and a resolution link.	LIVE
In-session distress rules	SUDS rated between every set. Hard stop at ≥9, pause at ≥8, rise-of-3 pause, 35-minute wind-down, 45-minute hard cap, 1 processing session per 24h, 24-hour cooldown after a high-distress ending.	LIVE
"Ground me" control	Fixed, always visible in-session. One tap, no confirmation, halts stimulation, pivots to grounding, logs a coded safety event.	LIVE
SOS panic panel	Persistent button on every screen, both platforms. Opens the member's own calm place, grounding tools, one-tap call to their named safe person, crisis line, and 911. Logs a coded event only — never content.	LIVE
Crisis routing	Region-aware resources, always reachable without login. A deterministic regex pre-filter runs <i>before</i> any model call and routes to a versioned scripted response — never model improvisation.	LIVE
Tamper-evident audit log	Append-only hash chain across identity, consent, clinical, module-runtime, specialist, billing, safety, and security events.	LIVE
Kill switches	Global new-session disable plus per-capability switches (generative conversation, provider sharing, escalation automation) — production-safe within minutes.	LIVE

3.2 Base layer — the daily habit (\$6.99/mo)

This is the acquisition engine and the retention engine. It is deliberately priced below every major competitor and deliberately generous.

Feature	Detail	Status
Breathwork — 5 patterns	Coherent breathing, longer exhale, physiological sigh, box breathing, 4-7-8. Visual pacer with haptics on iOS. No-breath-hold variants always available; holds are flagged.	LIVE
Meditation — 6 practices	Orienting to now, breath anchor, calm place, self-compassion, gentle body scan, containment. Trauma-informed: eyes-open options, explicit permission to skip or stop.	LIVE
Movement — 5 practices	Orienting turns, rooting down, easing tension, shaking it off (stress-cycle discharge), strong-and-contained pressing. Seated options throughout.	LIVE
Sleep — 4 wind-downs	Winding down, melting into rest, putting the day down, safe and warm. Slower pacing, no "return to the room" ending — each trails off into permission to sleep.	LIVE
Safety titration	On an elevated check-in day, gentler and no-hold practices are automatically surfaced first. The library reorders itself around the person's nervous system.	LIVE
Psychoeducation — 7 lessons	Window of tolerance, why the method works, understanding triggers, grounding the nervous system, titration & dual attention, self-compassion, building a calm place. Read-tracked, cross-linked to modules, surfaced inside sessions.	LIVE
Grounding + safety plan	Personalized page built from the member's own tools, reminder phrase, and named support contact.	LIVE
AI companion — 1x/week	Full companion access, capped at one conversation per week. Crisis messages are exempt from the cap, twice over. The cap is the primary, humane upgrade trigger.	LIVE

Total shipped guided content: **20 practices**, **~49 scripted minutes**, plus 7 lessons (~22 read-minutes). All deterministic text — no media pipeline required to ship, and narrated on-device with zero marginal cost per play.

3.3 Plus layer — the program that remembers you (\$19.99/mo)

Feature	Detail	Status
11-module clinical program	Calm place → containment → body scan → trigger map → resourcing → recent-trigger → safe-target → installation → future template → relational → maintenance. 5 are specialist-gated by design.	LIVE
Unlimited companion + memory bank	Long-term memory across 9 typed categories with provenance and graceful forgetting. Remembers triggers, grounding tools, focus areas, session patterns. Member-viewable, editable, deletable.	LIVE
Validated measures	PC-PTSD-5, PCL-5, ITQ (ICD-11 complex-PTSD criteria), PHQ-9, GAD-7. Baseline plus weekly cadence, trend charts, automatic worsening alerts.	LIVE
Care paths — 10 routes	Goal-based pathways (PTSD, anxiety & panic, phobias, recent event, grief, confidence, low mood, complex-trauma readiness, cravings, pain & somatic) each with an honest evidence grade and clinician-review level.	LIVE
AI program plan	Sequences the member's own triggers lowest-intensity-first, flags intensity ≥7 as specialist territory, with a deterministic rules fallback so a model outage never leaves a member planless.	LIVE

Feature	Detail	Status
Therapy knowledge base	16 modalities, 70 clinician-reviewable techniques, retrieved deterministically by tier / activation / dissociation state. Crisis tier receives none.	LIVE
Specialist review	Clinician console: alert queue, member detail, unlock decisions with documented reasons, consent and audit ledgers.	LIVE

3.4 Premium layer — Autopilot, the autonomous care loop (\$34.99/mo)

This is the tier that has no competitive equivalent. Plus *remembers* you between sessions; Premium *acts* between them.

Pillar	Detail	Status
1. Daily plan composer	Each morning, composes a concrete plan from that day's check-in, the program plan, titrated practice selection, and unread lessons. Recomposes the moment a check-in lands. Every session item passes the same server-side gate as the session player.	LIVE
2. Proactive outreach	The companion speaks first on coded signals: a 3+ day check-in gap, worsening measures (PCL-5 +10, ITQ +8, PHQ-9/GAD-7 +5), or a 5-practice week worth celebrating. Cooldowns and audit on every send.	LIVE <i>in-app; push pending</i>
3. Adaptive pacing, made visible	Every plan carries a note explaining what Autopilot adjusted and why — "sessions are set aside today; protecting your window is progress too." The engine's caution reframed as the benefit it is.	LIVE
4. Continuous risk watch	Worsening trends open an early clinician alert before the member has to ask. Coded types only, never content.	LIVE
5. Priority specialist review	Premium unlock requests queue at elevated severity. The <i>clinical bar for the decision is identical</i> on every tier — only the queue position differs.	LIVE
Live spoken sessions	Hands-free voice: the member speaks during a session without pressing anything; the system responds from memory, rules, and the therapy KB. The deterministic engine still owns every clinical decision.	GATED

The safety invariant that makes Autopilot defensible

Autopilot **only ever narrows a day**. It can make a day gentler; it can never make one riskier. Every session it proposes is validated against the same 14-step gate the member would hit manually, crisis days compose a support-only plan, and **no model call composes any plan** — it is fully deterministic and reproducible. This is the difference between "AI that decides your therapy" (uninvestable) and "automation of clinician-validated rules" (defensible).

3.5 Platform & infrastructure

- **Web application** — Next.js 16, server components, ~25,000 lines across 159 source files.
- **Native iOS app** — SwiftUI, 50 files / ~6,300 lines, full feature parity including SOS, all four practice libraries, lessons, companion, and the Autopilot plan.
- **Shared backend** — 29 mobile API endpoints; one memory bank, one safety engine, one source of truth. Calm place saved on web appears on the phone.

- **Encryption** — AES-256-GCM at the application layer for all member free text, memory values, and chat messages.
- **Privacy architecture** — no advertising trackers, no analytics pixels, no third-party data brokers. On-device speech synthesis and recognition; audio never leaves the device.
- **Test coverage** — 259 automated tests across 30 suites plus 17 end-to-end browser tests, including adversarial red-team harnesses. CI blocks on safety tests, accessibility (WCAG AA), dependency vulnerabilities, and a banned-marketing-vocabulary grep.

What it takes to make the top-tier items fully usable

The honest picture: **four of the five Autopilot pillars are live and working today.** What remains is a bounded, well-documented list — and critically, almost none of it is engineering risk. It is external review, external accounts, and signatures.

4.1 Live today — no further work required

- Autopilot daily plan composer, adaptive pacing, risk watch, priority review
- Proactive outreach (delivered in-app; the message is waiting the moment the app opens)
- All 20 practices, 7 lessons, SOS, the 11-module program, memory bank, measures, care paths
- Three-tier billing, the 7-day Premium trial, plan changes, the upsell engine

4.2 Gated — what unblocks each, who owns it, and what it costs

Capability	What unblocks it	Owner	Cost	Time
Push-delivered outreach <i>makes "reaches out" literal</i>	Paid Apple Developer account + APNs implementation	Founder → eng	\$99/yr + ~1 wk eng	1–2 weeks
Live spoken sessions	Clinician sign-off on the LIVE_SESSION_* register rows, then a config flag flip	Clinicians	Review session	1 session + flip
Autonomous governance <i>engine governs, not shadows</i>	(a) Independent privacy/security review, (b) human-factors testing, (c) marketing-claims rewrite, (d) counsel confirmation, (e) divergence-report walk	External + founder + counsel	~\$15–40k est.	6–10 weeks
Autonomous BLS <i>self-guided reprocessing</i>	Part-6 execution: BLS human-factors testing, red-team closure, staged Phase-4 rollout (cohorts 12 → 6 → 12), then a separate clinician sign-off	External + clinicians	~\$20–50k est.	3–6 months
Studio-quality voice	Founder decision: human voice actor vs. pre-rendered neural TTS; then record the fixed scripts	Founder	\$500–2k	1–2 weeks
Apple Watch + HealthKit	Apple Developer account; watchOS target; HealthKit consent flow	Founder → eng	Incl. above	4–6 weeks
Real payments	Stripe account with three prices + trial configured (integration seam already built)	Founder	% of revenue	Days
Scale infrastructure	Provision Postgres, one-time data load, raise instance count (code is complete and verified)	Founder	~\$7–25/mo	1 day

Cost estimates are engineering judgment for planning purposes, not quotes. Obtain real quotes before they enter a funding ask.

Why this list is a strength, not a weakness

Nearly every remaining item is **external validation, not construction**. The company is not asking for capital to find out whether the product can be built — it is built, tested, and deployed on two platforms. Capital buys the security review, the human-factors study, the clinical staged rollout, and the go-to-market. That is a materially lower-risk use of funds than "we will build it."

4.3 Sequencing — the critical path

1. **Weeks 0–2** — Apple Developer account, Stripe, email provider, auth provider. Unblocks push, real revenue, and the operational drills.
2. **Weeks 0–2 (parallel)** — Commission the independent security review and the UX-research human-factors study. These are the reviewers' explicit precondition and have the longest lead time — start them first.
3. **Weeks 2–6** — Clinician session A: accept the drafted evidence package, sign the live-session rows, walk the divergence report. Counsel engagement in parallel (three items, one retainer).
4. **Weeks 6–10** — External results land → clinician session B → claims rewrite → staged flag flip, one capability at a time.
5. **Months 3–6** — BLS Part-6 execution and the staged clinical rollout.

What sets Steady apart

5.1 Head-to-head

Capability	Calm	Headspace	Insight Timer	Sanvello	Steady
Relaxation / meditation content	Yes	Yes	Yes	Yes	Yes
Clinical intake screening with hard stops	No	No	No	Partial	Yes
Daily gate deciding what is safe today	No	No	No	No	Yes
Validated symptom measures + trends	No	No	No	Yes	Yes
Structured trauma-processing program	No	No	No	No	Yes
Deterministic in-session stop rules	N/A	N/A	N/A	N/A	Yes
Persistent clinical memory across sessions	No	No	No	Limited	Yes
Autonomous daily care loop	No	No	No	No	Yes
Licensed-clinician oversight in product	No	Separate paid service	No	Separate paid service	Yes
Tamper-evident clinical audit trail	No	No	No	No	Yes
Entry price	~\$14.99	~\$12.99	~\$9.99	~\$8.99	\$6.99

Competitor rows reflect publicly listed consumer plans and standard feature sets; re-verify against live pages before presenting.

5.2 The four defensible moats

1. The engine, not the content — and they are inseparable

A competitor can license 500 meditations tomorrow. What they cannot quickly obtain is a deterministic rules engine that has been reviewed and signed by licensed psychologists, with a documented conflict-resolution ledger and an auditable decision trail. In Steady, the content is worthless without the engine that decides *which* content this person gets *today*. That pairing took a clinical corpus, a rules architecture, and a signed clinical review.

2. Compounding memory as switching cost

Every conversation, trigger, calm place, and session outcome deepens a personal model the member can see, edit, and delete. Month 12 is meaningfully better than month 1 — and starting over elsewhere means starting from zero. This is the same retention dynamic that makes a personal CRM sticky, applied to something far more personal.

3. Structural AI safety — increasingly the licence to operate

The companion is *architecturally prevented* from making, reversing, or clearing any safety decision. A deterministic crisis filter runs before every model call. An output guard blocks a documented list of never-say outputs (simulated feelings, asserted internal states, diagnosis, cure claims, false monitoring claims, dependency-inducing language). As the category faces regulatory pressure over LLM-as-therapist, this architecture converts from cost centre to moat.

4. Cost structure

Twenty guided practices and seven lessons are **deterministic text**, narrated on-device. There is no per-play content cost, no CDN video bill, and no studio required to expand the library — a new practice is a writing session plus a clinician review. Competitors carry production and licensing costs that scale with catalogue size; Steady's marginal content cost is approximately zero.

The question every investor will ask: "Why won't Calm just add EMDR?"

They would need to: commission a clinical corpus; build a deterministic safety engine; obtain independent licensed-clinician ratification; accept liability for adverse events in trauma processing; add clinical screening that *turns away* paying customers (Steady auto-refunds anyone the screener excludes); and staff clinician oversight. That is a different company with a different risk appetite and a different regulatory posture — and it directly threatens the mass-market, low-liability positioning that makes their business work. The rational move for an incumbent is to **acquire**, which is itself the exit thesis.

How Steady makes money

6.1 The ladder

Tier	Price	Positioning	What unlocks
Base	\$6.99/mo	A calmer daily practice	Check-ins, all 20 practices, 7 lessons, grounding, SOS, companion 1×/week
Plus	\$19.99/mo	A program that remembers you	+ 11-module program, measures & trends, unlimited companion with full memory
Premium	\$34.99/mo	Steady runs your program with you	+ Autopilot, live sessions, priority specialist review

6.2 The trial mechanic — trial the top, then descend

Every new member starts with 7 days of Premium, free — regardless of which tier they select for billing. This is the single most important design decision in the monetization model, and it is already built.

- Every member *experiences* Autopilot, unlimited companion, and the full program before they pay anything.
- Members who fall back to Base or Plus have a concrete memory of what they gave up — the upsell references their own experience, not a feature list.
- The lower tiers are not a downgrade path; they are a **retention floor**. A member who would have churned at \$34.99 stays at \$6.99, remains in the funnel, and keeps building memory that makes upgrading more attractive later.

6.3 The upsell engine — revenue expansion as a product feature

Built and shipped. Recommendations fire only when the member's own behavior earns them:

Trigger	Recommends	Fires when
Weekly cap reached	Base → Plus	A Base member reaches out to the companion and the weekly window has closed. The most frequent, most natural trigger.
Trial win-back	Base/Plus → up	A recently-converted member is referenced back to what they had during the free Premium week.
Fit detected	Base → Plus	The member has named 2+ recurring triggers — exactly what the guided program addresses.
Strain detected	Plus → Premium	A hard-stopped session or 3+ elevated check-ins in 14 days — the member needs between-session support, which is what Premium is.

Discipline is enforced in code: a 5-day global cooldown, a 14-day per-type cooldown, Premium members are never pitched, and **nothing is ever attached to a crisis or risk-flagged exchange**. Every recommendation is logged for conversion analysis. The companion never becomes a salesperson — suggestions render as a distinct, dismissible card, visually separate from companion speech.

6.4 Revenue lines beyond consumer subscription

1. **Consumer subscription** — live today, three tiers.

2. **Annual plans** — not yet built; the standard category lever (~2 months free) for improving cash collection and reducing churn. Low engineering effort.
3. **Employer / EAP channel** — the clinician console, audit trail, and aggregate reporting are already built. Sold per-seat, this is the highest-ARPU channel in the category.
4. **Clinician-adjunct licensing** — therapists assign Steady between sessions and monitor via the existing console. Converts the clinical-oversight cost centre into a revenue line, and turns clinicians from skeptics into a distribution channel.
5. **Payer / health-plan** — long-horizon, requires outcomes data. The measures infrastructure (PCL-5, ITQ, PHQ-9, GAD-7 with trends) is already collecting exactly the evidence a payer contract requires.

The growth engine: low CAC, high retention, expanding ARPU

The strategy is one loop with three reinforcing stages. **Acquire cheaply at the bottom, retain structurally in the middle, expand revenue from behavior at the top.**

7.1 Stage one — the entry tier lowers acquisition cost

- **Price is the first CAC lever.** At \$6.99, Steady is the cheapest credible option in the category — roughly half of Calm and Headspace. Lower price means higher landing-page conversion, which directly divides blended CAC. The *same ad spend* buys more members.
- **The free Premium week removes the remaining risk.** The offer is not "start a trial" but "have all of it for a week, then pay as little as \$6.99." That is one of the lowest friction propositions in consumer health.
- **Cheap tiers make paid acquisition mathematically viable earlier.** A \$6.99 entry with strong retention can support paid channels that a \$34.99-only product cannot, because the payback period is driven by retention, not by first-month revenue.
- **Non-paid channels the product itself generates:** the psychoeducation lessons are genuine SEO surface area on high-intent searches ("window of tolerance," "why do I get triggered"); the always-free crisis and grounding pages attract high-intent traffic and build the trust the category badly lacks; and clinicians recommending Steady as homework is a zero-CAC channel that the console is already built to serve.

7.2 Stage two — the daily layer drives retention

This is why the practice suite matters commercially, not just clinically. **A trauma program alone is episodic — people engage in bursts and lapse.** A daily practice suite is a habit.

- **Daily reasons to open the app** — check-in, a two-minute breath, a wind-down at bedtime. Sleep content in particular creates a nightly ritual.
- **Titration removes the failure mode.** On a hard day, the app doesn't ask for something the member can't give — it offers something gentler. The most common churn moment in mental-health apps (feeling like you failed the app) is designed out.
- **Memory compounds.** Month 6 is better than month 1. Churn means losing it.
- **Autopilot removes the decision.** For Premium members the plan is already made — the highest-friction moment in any wellness app ("what should I do today?") is eliminated.
- **No dark patterns, deliberately.** No streaks, no guilt, no manufactured scarcity — these are explicitly banned in the safety rules. In trauma care, pressure tactics cause harm and churn; the trust this buys is the retention strategy.

7.3 Stage three — the upsell expands monetization

Once a member is retained cheaply, **revenue growth requires no additional acquisition spend.** The upsell engine converts usage into ARPU:

- A Base → Plus upgrade is a **2.9× revenue increase** on an already-acquired member.
- A Base → Premium upgrade is a **5.0× increase**.
- Marginal CAC on these upgrades is **zero** — the trigger is the member's own behavior.

Because the recommendation is earned and specific ("you've named a few triggers you keep running into — that's exactly what the guided program works through"), it converts better than a generic paywall and does not damage trust when

declined.

7.4 The unit-economics framework **MODEL — NOT RESULTS**

Steady has no production users. The following is the **model shape** the deck should present, with the assumption cells filled from real data once the pilot runs. Present these as scenario mechanics, never as performance.

Driver	Source	Why it matters
Trial → paid conversion	To be measured	Set by the Premium-week experience; the single highest-leverage metric in the model
Initial tier mix (Base/Plus/Premium)	To be measured	Determines entry ARPU; the ladder is designed so a Base-heavy mix is still healthy
Monthly churn by tier	To be measured	Expect materially lower churn on Base than category norms because of price and the daily habit layer
Upgrade rate / month	To be measured	Every upgrade is a step-change in ARPU at zero marginal CAC — this is the compounding term
Blended CAC by channel	To be measured	Organic (lessons/SEO, clinician referral) should carry a materially lower CAC than paid
Gross margin	Structural	Software margins: no per-play content cost, on-device narration, no human delivery capacity constraint

The mechanic to put on the slide

Most subscription businesses fight churn to protect a fixed ARPU. Steady inverts it: **acquire at a price low enough that retention is easy, then let ARPU rise over the member's lifetime as the product earns it.** Lifetime value grows with tenure rather than decaying — and the upsell trigger (a member reaching for support more than once a week) is also the clearest signal that the product is working for them. *Commercial interest and clinical interest point the same direction.*

7.5 Growth phases

- 1. **Phase 1 — Validate (0–6 months).** Small supervised pilot cohort. Prove retention, trial conversion, and safety-event rates. Complete the external gates. Turn on real payments and push.
- 2. **Phase 2 — Consumer scale (6–18 months).** Paid acquisition against a measured CAC, content/SEO engine, App Store presence, annual plans, Watch and HealthKit.
- 3. **Phase 3 — Channel expansion (18–36 months).** Employer/EAP with the existing console, clinician-adjunct licensing, and outcomes data assembled toward payer conversations.

Risks, and how each is already mitigated

Investors will find these anyway. Naming them first, with the mitigation already in the product, is stronger than being asked.

Risk	The concern	Mitigation in place
Clinical / adverse event	A member is harmed during self-guided trauma work	Mandatory screener with hard stops and auto-refund; daily gate; deterministic stop rules; one-tap Ground-Me; 24h cooldowns; specialist review; kill switches; adversarial red-team test harnesses. Autonomous reprocessing stays disabled pending staged clinical validation.
Regulatory	Classified as a medical device, or claims challenged	Deliberate wellness-lane posture; no diagnosis or treatment claims; CI blocks banned marketing vocabulary automatically; versioned consent ledger; counsel review on the critical path before any autonomy governs.
Professional-body friction	EMDR bodies discourage self-administration	Known and explicitly scoped to counsel. Stabilization and resourcing (the shipped core) are far less contested than reprocessing, which remains gated behind staged validation and separate sign-off.
AI safety	A model says something harmful	The model cannot make safety decisions by architecture. Deterministic crisis filter runs before every model call; output guard blocks a documented never-say list; scripted fallbacks; full rules-engine fallback if the model is unavailable.
Competitive	An incumbent adds trauma content	Requires clinical governance, liability appetite, and screening that turns customers away — contrary to their model. Acquisition is the rational incumbent response.
Concentration	Small team, key-person dependency	Fully documented: README as handoff spec, architecture decision records, go-live runbook, sign-off ledger, 259 tests encoding intended behavior. The system is transferable.
Execution	Remaining gates slip	Every gate has a named owner, a defined deliverable, and a written acceptance criterion in the consolidated sign-off checklist. Two are on the critical path and can start immediately.

Use of funds — what capital actually buys

Allocation shape for the deck; replace percentages with the real raise amount and quoted figures.

Category	Weight	What it delivers
Clinical validation & external gates	~20%	Independent security/privacy review, human-factors studies (session UI + BLS), staged clinical rollout, ongoing clinician oversight. <i>Unlocks the top tier and the autonomous claim.</i>
Go-to-market	~35%	Paid acquisition against a measured CAC, the content/SEO engine, App Store presence, clinician-referral channel development.
Product & engineering	~25%	Push notifications, Apple Watch, HealthKit, annual plans, produced audio, employer/console features, infrastructure scale-out.
Clinical & support staffing	~12%	Specialist review capacity as the member base grows; member support.
Legal, compliance & insurance	~8%	Counsel engagements, cyber-liability cover, ongoing compliance.

Milestones this capital reaches

- **Month 3** — external gates commissioned and running; real payments live; push shipped; pilot cohort recruited.
- **Month 6** — pilot retention, trial-conversion, and safety data in hand; autonomous governance flag flipped stage-by-stage; first real unit economics.
- **Month 12** — measured CAC and LTV; paid acquisition scaling against known payback; Watch and HealthKit shipped; annual plans live.
- **Month 18** — employer/EAP channel in market; clinician-adjunct programme; outcomes dataset accumulating toward payer conversations.

Deck-builder handoff

10.1 Suggested slide map

#	Slide	Source	The one thing it must land
1	Title + positioning line	Cover	"Runs a clinician-governed trauma program with you, daily, for less than streaming."
2	The problem	\$2	\$150–350/hr and months of waitlist, or a meditation app with no clinical structure. Nothing in between.
3	Market gap	\$2 table	Two poles, empty middle — with prices on screen.
4	What Steady is	\$3 overview	Three layers on one safety substrate. Show the product, not a diagram.
5	Live product demo	Demo site	It is real and running on web and iOS <i>today</i> .
6	The safety engine	\$3.1	30 deterministic rules, 14-step gate, ratified by two licensed psychologists. This is the asset.
7	Autopilot	\$3.4	No competitor has this. Plus remembers; Premium acts. Only ever makes a day gentler.
8	Competitive matrix	\$5.1	The row of "No / No / No / No / Yes ."
9	Moats	\$5.2	Engine+content inseparable; memory switching cost; structural AI safety; near-zero content marginal cost.
10	Pricing ladder	\$6.1	Three tiers, and everyone starts on Premium free for a week.
11	The funnel loop	\$7	Cheap entry → habit retention → earned upsell . Make this one diagram.
12	Upsell engine	\$6.3	2.9× / 5.0× ARPU at zero marginal CAC, triggered by the member's own behavior.
13	Unit economics	\$7.4	Model shape, clearly labeled as assumptions.
14	Traction & status	\$3, \$4.1	Built, tested, deployed, clinically ratified. Pre-revenue by choice — gates first.
15	Path to full activation	\$4.2	Remaining work is external validation, not construction.
16	Growth plan	\$7.5	Validate → consumer scale → channel expansion.
17	Risks & mitigations	\$8	Name them first; every one already has a mitigation in the product.
18	Use of funds & milestones	\$9	Capital buys validation and go-to-market, not discovery.
19	Team	—	<i>To be supplied — not covered in this document.</i>
20	The ask	—	<i>To be supplied — amount, structure, valuation.</i>

10.2 Design direction

- **Palette:** ground #2f3a33, olive #545e53, sage #a8b8a1, sage-deep #93a58b, ivory #f7f1e8, linen #fbf8f2, moss #dce4d4. Accents: support/red #9a4f42 (crisis only), pause/amber #d7a85f, safe/green #7fa37b.
- **Type:** serif for headlines (the product uses Cormorant Garamond), clean sans for body. Generous whitespace — calm is the brand.
- **Tone:** calm conviction. No hype adjectives, no exclamation marks, no stock photos of people staring wistfully out of windows. The restraint *is* the credibility.
- **Screenshots to feature:** the Autopilot plan card on the dashboard; the SOS panel; the breathing pacer mid-practice; the clinician review console; the check-in routing a hard day to grounding.

10.3 Language rules — non-negotiable

- **Never** use: cure, heal, treats PTSD, clinically proven, AI therapist, guaranteed results. These are blocked in the product by an automated CI check and must not appear in the deck either.
- **Always** say: "built on the EMDR method," "self-guided wellness program," "not therapy, not medical care." Research claims describe **the method**, never Steady's own outcomes.
- **Never** present the unit-economics model as achieved performance, or imply users exist. "Pre-revenue, built and clinically ratified" is a stronger and safer story than a vague implication of traction.
- **Always** note that autonomous reprocessing and full autonomous governance remain gated pending validation — the discipline is a selling point to any investor who has seen a digital-health blow-up.

10.4 Assets and where to get them

- **Live demo** — `steady-emdr-demo.onrender.com`. Sign in as the seeded Premium member to show the Autopilot plan as the dashboard centerpiece.
- **Product screenshots** — capture from the live demo (web) and Xcode simulator (iOS).
- **Clinical sign-off** — signed PDFs in `docs/autonomous/`; a redacted cover page makes a strong credibility slide.
- **Research citations** — the three RCT/meta-analysis sources with DOIs are on the live landing page and repeated in §2 of this document.
- **Full technical spec** — the repository README is the complete handoff document covering workflow, instruments, scoring, and gating.

Steady — Investor Deck Source Document · 29 July 2026 · Confidential. Product counts, test counts, and capability claims verified against the shipping codebase on the date of preparation. Financial figures are models with stated assumptions, not results. Competitor pricing reflects publicly listed plans and should be re-verified before presentation. Steady is a development prototype: not a medical device, not for emergency use, and not cleared for clinical use. Nothing in this document is an offer to sell securities.